



PAWSIC

POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

www.pawsic.org

STANDARDS & ASSESSMENT FRAMEWORK

2026 EDITION

Mobile Wound Group *Checklist*

A standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings.

SECTION III • SETTING-SPECIFIC ADD-ON

Home Health Agency *HHA*

Complete in addition to Section I: Core Standards.

What's *Inside*

- ✓ PDGM & OASIS Alignment HHA-1
- ✓ Visit Utilization & Cost Management HHA-2
- ✓ Field-Based Care Reality HHA-3
- ✓ Physician Order Workflow HHA-4
- ✓ Social Determinants of Health (SDOH) & Home Environment HHA-5

Scoring

- 0 Absent
- 1 Emerging
- 2 Established
- 3 Best-in-Class

★ Critical items are flagged throughout to signal elevated regulatory or clinical risk.

HOW TO USE

Complete Section I, then your *setting add-on*

Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV).

SECTION I
Core

SECTION II
SNF

SECTION III
HHA

SECTION IV
ALF/IL

Purpose

The PAWSIC Mobile Wound Group (MWG) Checklist 2026 is a standardized framework for evaluating, aligning, and optimizing mobile wound care practices across post-acute care settings. It is intended to support:

- Wound Providers (WPs) and Wound Provider Groups (WPGs): A tool to support program development or process improvement.
- Healthcare Leaders: A tool for any post-acute site of service to aid in the selection of a WP or WPG.
- Patients/Responsible Parties: A reference guide when selecting a WP or WPG.

Intent

This checklist is solely a guide and covers a variety of competencies for both employed WPs and contracted WPGs. It is intended to be adaptable to the needs specific to a site of service, regulations, and applicable practice acts. Regulations, reimbursement structures, and policies that direct practice and care differ from state to state, payer to payer, and by site of service.

This checklist is not intended to replace your facility/organization's policies and procedures nor any local, state, or federal requirements. The utilization of this form is at your discretion. PAWSIC releases this form to your facility/organization to assist in selecting or guiding a partnership with a WP/WPG.

AUTHORS

Pamela Scarborough, PT, DPT, CWS, FAWWC, Jeanine Maguire, PhD, MS, MPT, CWS, FCPP, Dr. Ziad Mirza, MD, Therese Laub, LPN, CWS, Julie Rhodovi, MBA, CWCA, LBBH, Cathy Milne APRN, WOCN, Dr. Ronald Rosen, MD, CMD, CWSP, Nicole Brandt, PhD, PharmD, Dr. Cassandre Voltaire, DO, ABWMS, Shawn Naqvi, MD, CEO/CMO

REVIEWERS

Ashley Amaireh, CNP-CWS; Melissa Ramdas, DNP, MS APN-C CCM CWCN-AP; Carl Tabor, RN, BSN, LNHA; Marc Krakow MD, FAAEM, CWSP, Elisha N. Fridley, P.C

Instructions: Complete Section I (Core) in full. Then complete only the Setting-Specific Add-On for your care setting (Sections II, III, or IV). Section V is a patient-facing document; clinical reviewers do not need to complete it.

★ **Critical items are flagged throughout the document to signal elevated regulatory or clinical risk requiring priority follow-up**

Assessment Intake

Complete this block before starting each evaluation. All fields are required for traceability.

FIELD	ENTRY	ADDITIONAL INFO
Facility / Organization Name (include prior company names, acquisitions, rebranding etc)		
WP / WPG Being Evaluated		
Reason for Review: (New group, annual review, post-incident, due diligence, etc.)		
WPG/WP patient volume/footprint (number of facilities/states)		
Reviewer Name, Title/Role		
Reviewer Disclosures (MWG competitor, vendor, former employee)		
Reviewer of Evidence Name, Title/Role		
Date of Assessment		
Care Setting (select one)	☐ SNF (Skilled Nursing Facility) ☐ HHA (Home Health Agency) ☐ ALF (Assisted Living) ☐ IL (Independent Living)	

SECTION III

Setting-Specific Add-On: HHA

Home Health Agency | Complete in addition to all Core sections (Section I) | Skip if care setting is SNF or ALF/IL

SCORING

0 Absent

1 Emerging

2 Established

3 Best-in-Class

★ Critical Items, these need to be summarized separately

Comments/Findings/Actions: Reviewer to complete

Evidence Reviewed: in some cases this may be N/A; in others, the evidence used to support findings should be validated through observation of a legal document, website review, etc.

HHA-1 PDGM & OASIS Alignment

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
WP training on OASIS wound items: etiology accuracy, item completion			

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Understanding of PDGM reimbursement categories and impact of wound documentation			
Collaboration with agency OASIS clinicians to ensure coding consistency			
WP documentation clarity to support billing under PDGM			

HHA-2 Visit Utilization & Cost Management

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Recommended visit frequency framework based on wound type and acuity			
Approach to HHA collaboration to ensure wound care consistency			
Supply cost awareness and transparency (burden to the agency)- WP policy on skin substitute / CTP use within the HHA episode (zero, restricted, or unrestricted)			
Process for recommending supply changes that affect agency cost			
Part A / Part B billing clarity: what the WP bills, what is excluded from the home health episode, and how the WP confirms no duplication			

HHA-3 Field-Based Care Reality

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Communication approach with field clinicians			
Escalation pathways for: infection, wound deterioration, acute changes			
Support for field clinician education (practical, in-home context)			
Adaptations for limited resources in the home environment			
Clear escalation protocols for HHA, non-clinical support including patient/family			

HHA-4 Physician Order Workflow

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Timeliness of verbal and written orders			
Collaboration with referring and attending physicians			
Documentation clarity and completeness for billing support			

HHA-5 Social Determinants of Health (SDOH) & Home Environment

CRITERION / CONSIDERATION	SCORE	COMMENTS / FINDINGS / ACTIONS	EVIDENCE REVIEWED
Assessment of caregiver support and capacity			
Consideration of access to wound supplies in the home			
Home environment safety assessment related to wound care			
Communication of SDOH findings into the care plan (health/wound literacy, food availability/affordability, pharmacy access, etc.)			

**PAWSIC**

POST-ACUTE WOUND & SKIN INTEGRITY COUNCIL

Post-Acute Wound and Skin Integrity Council (PAWSIC) is a non-profit professional group advocating for best practices in wound care and patient-centered care in the post-acute medical sector.

Copyright © 2026 PAWSIC. All rights reserved.